SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

# Lomustine + bevacizumab 5 mg/kg

NCCP 00742: recurrent malignant glioblastoma; bevacizumab use is off-label. · NCCP 00742 · version 3 · 42-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
Lomustine chemotherapy and bevacizumab targeted therapy are used together to help control recurrent glioblastoma. Discuss the expected benefit, symptom goals and treatment burden with your team.

**Schedule**
Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1, 15 and 29. Up to six cycles are planned, with review before each treatment.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

### Lomustine
ORAL CHEMOTHERAPY CAPSULE

- ☐ Delayed and cumulative low blood counts
- ☐ Nausea / vomiting
- ☐ Lung injury
- ☐ Kidney and liver changes
- ☐ Rare second cancers and fertility effects

### Bevacizumab
TARGETED ANTIBODY GIVEN INTO A VEIN

- ☐ High blood pressure and rare PRES neurological syndrome
- ☐ Protein in the urine and kidney injury
- ☐ Bleeding, including serious internal bleeding
- ☐ Blood clots, stroke or heart attack
- ☐ Slow wound healing and wound complications
- ☐ Gastrointestinal perforation or fistula
- ☐ Infusion reactions and rare severe allergy
- ☐ Fertility effects and harm to a pregnancy

Discuss the exact diagnosis, treatment goal, evidence limits, alternatives and burdens; medicine-specific risks, monitoring, fertility and pregnancy precautions; any unlicensed supply or off-label use; and practical support. Keep steroid and seizure plans separate from anticancer treatment.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00742 v3.

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